

STATEMENT OF WORK

The purpose of this solicitation is to obtain offers from contractors who can provide temporary housing in community-based Contracted Emergency Residential Services (CERS). The contractor shall provide case management services to Veterans with the goal of permanent housing. Length of stay and service typically ranges from 30 to 90 days, not to exceed 180 days, therefore finding permanent housing is the primary goal of veterans in this program.

The successful contractor shall furnish transitional housing for the beneficiaries of the Department of Veterans Affairs Medical Center adjacent to the VA Health Care System located on 1101 Veterans Drive, Lexington, KY 40502-2235. The contractor shall provide safe housing, food, and provide support to each homeless Veteran related to "Activities of Daily Living" (ADL's). Contract is for a Base Year plus Four-Years.

A. PURPOSE

The Department of Veterans Affairs Medical Center ("VA") requires contractors to provide services as part of its Community Based Health Care for Homeless Veterans (HCHV) program. The goal of the HCHV program is to remove homeless Veterans from the street or other habitations unfit for Veterans and place them in community-based, residential environments with sufficient therapeutic services to meet the needs of each Veteran. The contractor will provide case management to help support the main goal of permanent housing along with secondary goals as clinically indicated.

B. BACKGROUND

The HCHV program is vital for providing a gateway to VA and community-based supportive services for eligible Veterans who are homeless. This includes ensuring that chronically homeless Veterans or Veterans experiencing homelessness can be placed in community-based programs that provide quality housing and support services. Through the HCHV program, VA provides support services to Veterans and facilitates their ongoing access to a broad range of medical, mental health, and rehabilitative services.

The contractor's facilities shall provide transitional housing. The price for these services must be based on an all-inclusive rate and must be located within .3 miles of a Lextran KY bus stop. The contractor shall furnish services only to VA beneficiaries from whom such care is specifically authorized by the Contracting Officer's Representative (COR) at the VA Health Care System, Lexington, KY. It is understood that the type of Veterans to be cared for under this contract will normally require direct care and personal services over and above basic room and board. Homeless Veterans frequently suffer ongoing mental illness, substance abuse, and other medical issues. The contractor will be required to provide a low-barrier therapeutic milieu and attendant services targeting the underlying factors contributing to each individual's homelessness. The facility will not be used for detoxification or other hospital level treatment.

The Facility must be compliant with The Americans with Disabilities Act (ADA) for at least six residents, along with meeting all other applicable state and federal regulations applicable to residential programs.

C. REQUIREMENTS

The contractor shall furnish each Veteran authorized care under this contract with the following basic services:

- a. Full room and board- Including adequately sized room with a bed, mattress, pillow, bed linens (Including sheets, pillowcases, and mattress cover) along with other appropriate furnishings, including a dresser and storage. Bathroom shower and all personal care necessities, including but not limited to towels, washcloths, toilet paper, toothbrush, soap, toothpaste, lotion, shaving materials, and menstruation supplies. The contractor may choose to provide accommodations where veterans share bedrooms, bathrooms and living spaces.
- b. The contractor shall provide all necessary food and drinks each day to provide 3 full meals per day, 7-days per week which meet the FDA's Dietary Guidelines for Americans 2020-2025, released December 2020. In addition to the daily meals, the contractor shall ensure the ongoing availability of a variety of nutritious snacks between meals and at bedtime for those requiring or desiring additional food, when it is not medically contraindicated. The addition of nutritious snacks to the requirements for room and board is particularly indicated for homeless patients. Many of these Veterans are either undernourished or have developed poor eating habits or both, often due to chronic psychiatric disorder and/or substance abuse behaviors. (The local VA dietitian may consult with the initial inspection team and the team making subsequent assessments and may evaluate not only the printed weekly/monthly menus but also the Veteran's satisfaction with meals and the actual consumption of food offered. Menus must be submitted monthly, not less than 10 business days before month's end for the coming month to the COR electronically.) For definition of "nutritionally adequate" and guidance on amounts of each food group per Veteran provided, see <http://www.choosemyplate.gov/>
- c. Access to a clean refrigerator and microwave.
- d. Contractor will develop a receipt of food provided to the Veterans and have it signed upon delivery by one resident, with a basic list of what was provided. This receipt will be presented upon request by the COR/Case Manager.
- e. Laundry facilities, including a washer and dryer, for residents to do their own laundry or to have laundry done, which shall include baskets, laundry detergent and dryer sheets/fabric softener.
- f. The number of beds required is a minimum of 12 and not to exceed 20. The contractor must have 12 beds available, at all times. The contractor may choose to include 4 beds that are able to be utilized by veterans who are on the sex offender registry, but not exclusive to those veterans. Contractor may utilize a secondary location for those 4 beds if necessary but not required. Contractor is not required to provide 4 beds for veterans on the sex offender registry, but it is preferred.
- g. Case management service goals should be clearly documented in an Individual Service Plan (ISP) developed by the contractor with input from the Veteran and the VA HCHV caseworker or designee. For each individual resident based on that resident's individual needs and interests. The ISP must include a screening of each veteran "Social Determinants of Health". The Social Determinants of Health screening results, along

with the Veteran's medical needs, shall inform the ISP. The ISP will be developed within 72 hours of admission.

- h. The ISP shall include permanent housing goals and objectives to reach the goal. It is the responsibility of the contractor to assist the Veteran in their housing search and to make referrals to the community to obtain housing. A daily individual progress report detailing the health status, unique emergent needs and progress activities for each Veteran shall be maintained (signed and dated by individual's caregivers on that date). There will be an expectation that proper daily documentation verifying services and case management efforts with the caregiver's name shall be placed in each Veteran's secure personal file. The notes are to be written professionally in a format that comports with the Social Determinants of Health and the Veteran's unique medical considerations.
- i. Services, which the contractor shall include:
 - A comprehensive Individual Service Plan (ISP):
 - Structured group activities and individual instruction must be provided daily and documented in the Veteran's "Daily Activity Record" of services provided.
 - A "Daily Activity Record" which is developed based on the ISP goals shall be maintained for each participating Veteran and shall be signed by the individual who provided each day's supports and services. Examples include but are not limited to group therapy, social skills training, emotional skills development, Alcoholics Anonymous, Narcotics Anonymous, Vocational Counseling, Occupational Training and Physical Activities.
- j. Every other week a collaborative meeting with VA program staff shall be held to review each Veteran's work and progress, to include all elements listed in letter "h" above as well as supportive psychosocial services and review of goals from the ISP.
- k. The Contractor's staff shall provide each Veteran with assistance to develop responsible living patterns, to maintain an acceptable level of personal hygiene and grooming, and to achieve a more adaptive level of psychosocial functioning, improved social skills, and improved personal relationships.
- l. The Contractor shall provide support to each Veteran as they adopt an alcohol/drug abuse-free lifestyle provided in an environment conducive to social interaction, group support work and the fullest development of the resident's rehabilitative potential.
- m. Each veteran shall be provided daily assistance to develop and to apply knowledge of their unique illness/recovery process in an environment inclusive of supportive recovery models.
- n. The Contractor shall provide a program that promotes regular typical community interaction.
- o. New residents shall be provided upon admission with an orientation to the facility, its programs, and daily routines. Orientation shall include Showing Veterans where laundry services are located, introduction to facility staff, dissemination of important phone numbers to contact the Program Manager and a phone to be used by Veterans to talk with family and friends. Program requirements and community rules shall also be

disseminated and carefully reviewed and explained (e.g., no alcohol/drugs on grounds, no violence, no threats, no stealing, etc.), how a Veteran will receive food during mealtimes and at other times, location of supplies, bedding, towels, transportation, and a discussion of the Veteran's part in participating in treatment services.

- p. Develop clear rules regarding behavior of residents with an avenue to discuss any disputes. Written residential rules are to be developed by the contractor and reviewed and agreed upon by VA contract staff.
- q. Ensure all house rules are in writing. Language must be in English and Spanish for explanations during orientation.
- r. A Pest management plan shall be developed and submitted which includes the following specifications: A scheduled plan of routine pest management, a monthly treatment schedule, routine prevention measures, and responsibilities of the contractor to address pest control issues immediately when they arise.
- s. The Contractor shall designate a staff member(s) to enroll and complete the Homeless Management Information System (HMIS) training. Designated staff person will enter Veteran information as required into HMIS database (Once Veteran signs Release of Information).
- t. Contractor must participate in Continuum of Care and be an active member of the Continuum of Care Team.
- u. Residents will be made aware of the use of security cameras to include residents signing documentation that explains what rooms have security cameras are monitoring/recording with residents signing the document acknowledging their awareness. Sleeping areas are prohibited from having cameras monitoring/recording.
- v. In order to enter transitional housing, veterans will sign documentation, agreeing to participate in random drug screens and breathalyzers as needed. If a veteran refuses to participate in a random drug screen or breathalyzer, this may prompt their discharge from transitional housing. If the results of a drug screen or breathalyzer indicate a positive for substance use, this may result in discharge from the transitional housing.

D. PERSONNEL

The Contractor shall employ adequate personnel to carry out the policies, contract responsibilities, and the program for the facility and shall submit a current staffing plan 10 days before commencement of services and monthly thereafter. The contractor must have appropriate and adequate staff coverage using authorized and unauthorized leave (e.g., holidays, sick leave, family care, etc.). The Ratio of case manager to Veterans is 16:1. Case manager should have at a minimum, a bachelor's degree in a Human services or closely related field. Case manager standards are organized around key attributes of responsible practice: Advocacy, Accountability, Professionalism, and Facilitation. In addition to the case manager there must be at a minimum, one staff member on duty on the premises or residing at the facility and available for emergencies 24 hours a day, 7 days a week.

On call staff and all residential staff shall complete training on crisis management, substance abuse, and mental health issues on a yearly basis. In addition, they must complete and maintain First Aid/CPR certification.

Background checks and resumes shall be completed and given to the Contracting Officer or the Contracting Officer's Representative (COR) before staff provide services to Veteran residents. During the first 90 calendar days of contract performance, the contractor shall make NO substitutions of key personnel unless the substitution is necessitated by illness, death, or termination of employment. Within 14 days after substitutions, necessitated by the emergency described above; the contractor shall provide background checks for the substitute key personnel. For substitutions proposed by the contractor after the initial 90 calendar day period, the contractor shall provide background checks and resumes for the substitute personnel, together with any other additional information requested by the Contracting Officer, at least 15 days before the substitution is to occur. The Contracting Officer shall notify the Contractor within 15 calendar days after receipt of all required information whether the VA shall approve the substitute key personnel. The VA reserves the right to refuse or revoke acceptance of key personnel if personal or professional conduct, or lack of required skills or experience, could jeopardize patient care or interfere with the regular and ordinary operation of the facility.

Temporary substitutions of key personnel shall be permitted in accordance with the contractor's written contingency plan. The contractor's contingency plan will be utilized if personnel leave the contractor's employment or are unable to continue performance in accordance with the terms and conditions of the contract and must be submitted as part of the Contractor's offer. This plan shall be reviewed and updated quarterly, at minimum.

E. PROGRAMING

Programing requirement: Contractor's staff are encouraged but not required to provide 2 hours of programing each week (Monday – Friday), non-holidays. In addition, each resident will be given 30 minutes of individual time a week with staff, or more as clinically indicated. This individual time will encompass training related to life skills development, which could include, but not limited to: food prep, resume development, job interviewing, professionalism in the workplace, substance abuse recovery if needed, and personal budgeting based on the individualized service plan of the Veteran. Staff may schedule and accompany or arrange transportation for applicable residents to local 12 step meetings. Staff are encouraged to bring 1-3 Veterans on grocery shopping trips, when needed, to teach Veterans shopping skills and nutrition planning.

Staff are to assist Veterans as needed to make nutritious meals, maintain a clean residence, and doing laundry; giving only as much assistance as needed to Veterans to complete these tasks. This can be part of their individual time with the Veterans. It is also encouraged that staff have a community meeting once a week to address any issues that the Veterans have, such as other community members, rules, pest management, etc.

Normal programing can be suspended during the following National Holidays and appropriate social activities substituted instead:

New Year's Day
Martin Luther King Jr's
Birthday
President's Day

Memorial Day
Juneteenth
Independence Day
Labor Day

Columbus Day
Veterans Day
Thanksgiving Day
Christmas Day

F. REFERRALS

Personnel authorized to refer Veterans for service shall make requests for service via telephone. The VA is responsible for determining eligibility of Veterans prior to referral to the contractor for services.

Orders, requests, or changes shall only be made by the Homeless Programs Manager, or authorized party specified by the Chief of Mental Health. Under no circumstances shall there be any deviation from any other party. In addition, contractor shall make no deviation and is responsible for providing directions to its staff.

A list of VA authorized ordering personnel shall be made available upon award of the contract. Employees may be added or deleted from the list during the term of the contract. The service provider shall be notified of such changes as they occur in writing.

It is understood that the contractor will not be paid for care provided to a referred Veteran beyond the period authorized in the referral, unless an extension of the authorization is provided in writing by the VA Contracting Officer.

G. ABSENCES AND CANCELLATIONS

The contractor shall notify the authorizing VA of unauthorized absences from the facility. Absences of the patient from the facility in excess of 48 hours will not be reimbursable except those with the prior approval of the VA facility liaison. Should a Veteran referred to a residential treatment facility/HCHV contract housing program, absent himself/herself in an unauthorized manner payment for services for that Veteran to the contract facility would be continued for a maximum period of two days provided there is an active outreach attempt on the part of the contract facility staff to return the Veteran to the residential treatment program and a strong likelihood that the patient will return. Management of program dropout will be an element of quality assurance review of this program. See attached Quality Assurance Surveillance Plan (QASP).

VA reserves the right to remove any or all Veterans from the facility at any time without additional cost, when it is determined to be in the best interest of the VA or the Veteran.

Length of stay (LOS) under this contract is dependent upon the needs of the Veteran as mutually determined by the Veteran, the residential staff, and VA-HCHV Staff. Maximum LOS will not exceed 180 days per admission with a maximum of one admission per FY without preapproval by VA.

The contractor shall notify the authorizing VA facility immediately when a medical emergency occurs that requires hospitalization of a referred Veteran. It is agreed that the Veteran will be admitted to the appropriate VA facility. When such admission is not feasible because of the nature of the emergency, it is agreed that hospitalization in a non-federal hospital may be necessary for protection of life. If hospitalization of a non-emergency nature is required, it is agreed that admission to the appropriate VA facility will be accomplished promptly.

The contractor shall notify the authorizing VA facility immediately of any incidents involving Veterans residing in the residential program. The contractor shall notify the VA case manager

by telephone during the hours of 8:00am and 4:30pm. For all incidents that occur after normal business hours, the contractor should notify the Administrator on Duty (AOD) at 859-281-4950. The contractor shall provide the HCHV case manager and the COR with a copy of the incident report within 24 hours. The contractor shall maintain a copy of the incident report in the Veteran's case record. Management of incidents and incident reporting, medical and otherwise, will be an element of quality assurance review of this program. See attached QASP.

H. CONDUCT

The contractor shall make available to the VA documentary information deemed necessary by the VA to conduct utilization review audits for the mandated national evaluation study as required by Section 2 of Public Law 100-6; to verify quality of care for Veterans, to assure confidentiality of Veteran record information and to determine the completeness and accuracy of financial records.

The contractor shall conduct treatment and discharge planning reflecting a team assessment of health, social and vocational needs and the involvement of resident and appropriate community resources in resolving problems and setting goals.

The contractor shall comply with the principles listed in 38 CFR I 7.707(b) to provide housing and supportive services in a manner that is free from religious discrimination.

Contractor personnel shall be expected to treat referred Veterans with dignity and respect and abide by standards of conduct mirroring those prescribed by current federal personnel regulations. The contractor shall comply with the VA Patient's Bill of Rights as set forth in 38 CFR 17.34a (copy available upon request).

The VA reserves the right to exclude contractor staff members from providing services to Veterans under this contract based on breaches of conduct, including conduct that jeopardizes patient care or interferes with the regular and ordinary operation of the facility. Breaches of conduct include intoxication or debilitation resulting from drug use, theft, patient abuse, dereliction or negligence in performing directed tasks, or other conduct resulting in formal complaints by Veterans or other staff members to designated government representatives. The contractor and COR shall deal with issues raised concerning contractor personnel conduct. The Contracting Officer shall be the final arbiter on questions of acceptability and validating complaints.

The following acts are not permissible, which provide services under this contract or while on facility premises:

- Use of intoxicating liquors, narcotics or controlled substances of any kind (excluding doctors' prescriptions which do not impair driver's driving ability) while on duty or reporting for duty while under the influence of liquors, narcotics or controlled substance of any kind (excluding doctors' prescriptions which do not impair driver's driving ability).
- Gambling in any form.
- Carrying of knives (blade length in excess of 2.5 inches), pistols, firearms or concealed weapons (including pepper spray and mace).

- Smoking in residence or while working with residents and other uses of tobacco while on duty.
- Resorting to physical violence to settle a dispute with a fellow employee, customer(s) or the general public while on duty.
- Spitting in prohibited places or any other unsanitary, offensive or insensitive practices or behavior.
- Use of loud, indecent or profane language and/or making threatening or obscene gestures toward customers or other employees.
- Stopping for personal business, including use of restroom facilities, while vehicle is occupied by a passenger. The driver shall not leave the vehicle with the key in the ignition at any time.
- Engaging residents in a verbal confrontation in an attempt to settle a disagreement. Should a disagreement arise, the driver is to contact his dispatcher/supervisor via the radio system or cellular telephone.
- Soliciting or accepting money from residents, companions or others at any time.
- Withholding food or supplies.

I. TRANSPORTATION

The contractor shall assist the Veterans with local transportation to scheduled meetings and appointments, which could include giving each resident a monthly bus pass while participating in the program. The contractor will be expected to help the Veteran access public transportation, including providing information and instructions necessary to enable Veterans to utilize public transportation. If VA staff determine that adequate public transportation is not available or appropriate for a Veteran, the contractor shall arrange for alternative transport by car.

J. FACILITY/SAFETY

It is the responsibility of the contractor to properly maintain its facilities and the VA shall have no responsibility for paying or reimbursing the contractor for such expenses. The contract facility must:

- i. Have a current occupancy permit issued by the local and state governments in the jurisdiction where the facility is located.
- ii. Be in compliance with existing standards of State safety codes and local, and/or State health and sanitation codes.
- iii. Be licensed under State or local authority.
- iv. Where applicable, be accredited by the State.

- v. Be equipped with operational air conditioning /heating systems.
- vi. Will assist Veterans in maintaining cleanliness of space on a regular basis, at least once a week or as indicated. Area must be kept free of dirt, grime, mold, or other hazardous substances and damage that noticeably detract from the overall appearance.
- vii. Supply each household dish soap, dishcloths or sponges, dish towels, window cleaner, general household cleaner, toilet cleaner, toilet brush, plunger, tub cleaner, and any other item to maintain cleanliness of the household.
- viii. Be equipped with first aid equipment and an evacuation plan in case of emergency. Have emergency contact phone numbers posted in plain sight in each facility.
- ix. Have windows and doors that can be opened and closed in accordance with manufacturers' standards.
- x. Provide basic furnishings like beds for each resident, dresser, lighting, living room seating for at least all residents in the residence, drying rack, silverware, cups, refrigerator, stove, television, cooking utensils, dining room table with chairs. Each Veteran must be supplied with a locked safe/cabinet that cannot be easily removed from the area for the Veteran's prescription drugs

The contractor facility must meet fire safety requirements, as follows:

- i. The building must meet the requirements of the applicable residential occupancy chapters of the current version of NFPA 101, National Fire Protection Association's Life Safety Code. Any equivalences or variances must be approved by the appropriate Veterans Health Administration Veterans Integrated Service Network (VISN) Director.
- ii. All residents in the facility must be mentally and physically capable of leaving the building unaided, in the event of an emergency.
- iii. Fire exit drills must be held at least quarterly. Residents must be instructed in evacuation procedures when the primary and/or secondary exits are blocked. A written fire plan for evacuation in the event of fire shall be developed and reviewed annually. The plan shall outline the duties, responsibilities and actions to be taken by the staff and residents in the event of a fire emergency. This plan shall be implemented during fire exit drills.
- iv. Plans for facility(ies) response to tornados and power outages will be written and reviewed with staff upon hire and annually.
- v. A written policy regarding tobacco smoking in the facility shall be established and enforced.

- vi. Portable fire extinguishers shall be installed at the facility. Use NFPA 10, Portable Fire Extinguishers, as guidance in selection and location requirements of extinguishers.
- vii. Requirements for fire protection equipment and systems shall be in accordance with NFPA 101. All fire protection systems and equipment, such as the fire alarm system, smoke detectors, and portable extinguishers, shall be inspected, tested and maintained in accordance with the applicable NFPA fire codes and the results documented.
- viii. The annual inspection by a VA team shall include a fire and safety inspection conducted at the facility.
- ix. All Department of Veterans Affairs reports of inspection of residential facilities furnishing treatment and rehabilitation services to eligible Veterans shall, to the extent possible, be made available to all government agencies charged with the responsibility of licensing or otherwise regulating or inspecting such institutions.

K. POLICY

The facility agrees and warrants that it does not maintain nor provide dual or segregated patient facilities, which are segregated on the basis of race, creed, color or national origin. The facility may neither require such segregated use by written or oral policies nor tolerate such use by local customs. The term "facilities" shall include but are not limited to rooms, wards, sections, eating areas, drinking fountains, entrances, etc.

The facility agrees to accept referral of and to provide all services specified in this contract for any person determined eligible by the VA Medical Center Director or his designee, regardless of race, color, religion, sex or national origin of persons for whom such services are ordered. In addition, the facility warrants that subcontracting will not be resorted to as a means of circumventing this provision.

It is agreed that the VA readily have access to all records concerning the Veteran's care in the facility. It is agreed that duly authorized representatives of the VA will provide follow-up supervision visits to Veterans placed to assure the continuity of care and to assist in the Veteran's transition back into the community. It is understood that these visits do not substitute nor relieve the facility in any way of the responsibility for the daily care and treatment of the Veteran. Upon discharge or death of the Veteran, records on all VA beneficiaries will be retained by the facility for a period of at least three years.

The facility will not accept food stamps or welfare from Veterans.

The facility warrants that all applicable fire laws are being complied with and there are no recommendations of fire officials, which have not been resolved.

The contractor shall comply with the VA Patient's Bill of Rights as set forth in the Code of Federal regulations, Section 17.33a, Title 38 (copy is available upon request).

The VA case manager/treatment teams, in collaboration with the existing community homeless care task force workers, shall identify and refer all potential residents to the contractor. All Veterans will be homeless.

Before referring any Veteran, the VA shall have provided the contracted agency with a signed Release of Information (ROI) from the Veteran as well as that Veteran's clinical assessment, physical and laboratory studies, by VA staff.

L. INSPECTION AND ACCEPTANCE

It is agreed that the Veterans Affairs will have right to inspection of the residential treatment center and all appurtenances by an authorized representative(s) designated by the Veterans Affairs.

Prior to the award of a contract and annually during the contract term, a multidisciplinary Veterans Affairs team consisting of a social worker, dietitian, registered nurse, and an engineering service safety officer and any other subject matter experts determined necessary by the medical center or HCHV Coordinator shall conduct a survey of the contractors' facilities to be used to provide food, shelter, and therapeutic services to assure the facility provides quality care in a safe environment. Inspections may also be carried out at such times as deemed necessary by the VA. **See attached inspection checklist.**

The contractor will be advised of the findings of the inspection team. If deficiencies are noted during any inspection, the facility will be given a reasonable time to take corrective action to notify the Contracting Officer that the corrections have been made. A written plan of action to correct deficiencies will be sent from the contractor to the contracting officer and COR within five days. A contract will not be awarded until noted deficiencies have been eliminated.

Failure by the Contractor to take corrective action within one (1) weeks' time will be reported to the VA Contracting Officer. If corrections are not made to the satisfaction of the VA, the Contracting Officer will consult with the appropriate officials so that suitable arrangements can be made to discontinue plans to award a contract, or to discharge or transfer patients and to terminate the existing contract, as appropriate.

The inspection team will focus on an assessment of the quality of life within the residential treatment facilities, giving particular attention to the following factors:

- General observation of residents indicates that they maintain an acceptable level of personal hygiene and grooming.
- The facility meets applicable fire, safety and sanitation standards in attractive surroundings conducive to social interaction and the fullest development of the resident's rehabilitative potential.
- The facility should be in a central location, near public transportation, and near areas which provide employment.

- Appropriate organized activity programs during waking hours (including evenings) reflecting a high level of activity in the facility or in the linked facilities, for example case management, physical activities, assistance with health and personal hygiene.
- There is evidence of facility-community interaction. This may be demonstrated by the nature of scheduled activities or by information about resident flow out of the facility, e.g., community activities, volunteers, local consumer services, etc.
- Staff behavior and interaction with residents convey an attitude of genuine concern and caring.
- Appetizing, nutritionally adequate meals are provided in a setting, which encourages social interaction and nutritious snacks between meals and bedtime are available for those requiring or desiring additional food, when it is not medically contraindicated. Making a spot check of Veterans' records to ensure accuracy with respect to Veterans' length of stay and services provided to the Veterans.

M. MEDICAL RECORDS

The contractor will not provide health care to patients.

The Certification & Accreditation of contractor systems requirements do not apply, and a Security Accreditation Package is not required.

N. PERFORMANCE STANDARDS

The contractor, along with the HCHV liaison, will work towards meeting targeted standards set for permanent housing and negative exits.

1. Percent of exits to permanent housing, 55% or greater. Numerator will be Veterans who exit directly to permanent housing and upon leaving CERS.
2. Percentage of Veterans with a negative exit of 20% or less. Numerator will be exits in which the Veteran was asked to leave because of violation of program rules (excluding exits due to threatened/actual violence to self or others), failure to comply with program requirements, or left the program without consulting staff.

Exclusions for both measures will be:

- Veteran has a Length of Stay (LOS) of seven days or less.
- Veteran is deceased at discharge.
- Veteran discharged due to threatened/actual violence to self or others.
- Veteran is transferred to another residential program.
- Veteran is placed into a hospital, long-term care facility or nursing home.
- Veteran is transferred to another HCHV CERS program due to need for more intensive care.

The Government performs surveillance to determine if the contractor exceeds, meets, or does not meet these standards. The Government shall use the standards below to determine contractor performance and shall compare contractor performance to the Acceptable Quality

Level (AQL). A Quality Assurance Surveillance Plan (QASP) was developed to measure this. **See attached QASP.**

O. QUALITY PERFORMANCE STANDARDS

The VA reserves the right to perform periodic and unannounced inspections. An initial and extensive (comprehensive) annual inspection will be performed by a team from VA service programs such as Facilities Management, Nursing, Police, and Social Work. Annual inspection(s) may be performed with/or in liaison with Homeless Programs or HCHV liaisons. Inspection(s) will include record reviews.

A VA inspection team will provide periodic inspections and any required follow-up inspections as needed.

The contractor must meet 100% of standards. The minimum standards are "Board and Care Standards" for inspections.

In addition, at a minimum, every two months a facilities check will be completed by the COR and the caseworker checking items, such as cleanliness of facilities, mold, bedbugs, damage to the facility, etc. A copy will be given to the contractor with any items that need to be addressed. Once a year, an inspection will be completed by the VA, separate from the normal facility inspections. **See attached QASP.**

The contractor will receive a copy of the inspection report and must correct deficiencies or provide an acceptable plan of action approved by the COR within 5 calendar days of notice. **See attached HCHV Contract Residential Inspection Checklist.** A sample inspection form is attached. This is a sample only for cursory review of possible inspection data. The VA may use updated and/or various inspection forms to complete the review.

Timelines and Penalties:

There will not be a deficiency notice of past performance issues if contractor performs correction within five (5) calendar days of notification of deficiency. Any life-threatening deficiencies must be corrected immediately. A re-inspection to ascertain corrections may be performed. If deficiencies continue, a deficiency and/or termination notice may be issued by the Contracting Officer.

Payment will be made for services rendered. Deficiencies may result in negative performance evaluation and may result in termination.

Deficiencies will be corrected within ten (10) calendar days of the deficiency, unless extended by the COR/CO. A plan of action to correct deficiencies that take longer than 10 days will be submitted to the COR.

P. RECORDS

The C&A requirements do not apply, and a security accreditation package is not required. Any documentation related to VA sensitive information (to include Veteran records) will be kept in a locked cabinet in a locked secure area.

RECORDS MANAGEMENT OBLIGATIONS

A. Applicability

This clause applies to all Contractors whose employees create, work with, or otherwise handle Federal records, as defined in Section B, regardless of the medium in which the record exists.

B. Definitions

"Federal record" as defined in 44 U.S.C. § 3301, includes all recorded information, regardless of form or characteristics, made or received by a Federal agency under Federal law or in connection with the transaction of public business and preserved or appropriate for preservation by that agency or its legitimate successor as evidence of the organization, functions, policies, decisions, procedures, operations, or other activities of the United States Government or because of the informational value of data in them.

The term Federal record: includes Lexington VA Health Care System (LVAHCS) records does not include personal materials. Applies to records created, received, or maintained by Contractors pursuant to their LVAHCS contract. May include deliverables and documentation associated with deliverables.

C. Requirements

Contractor shall comply with all applicable records management laws and regulations, as well as National Archives and Records Administration (NARA) records policies, including but not limited to the Federal Records Act (44 U.S.C. chs. 21, 29, 31, 33), NARA regulations at 36 CFR Chapter XII Subchapter B, and those policies associated with the safeguarding of records covered by the Privacy Act of 1974 (5 U.S.C. 552a). These policies include the preservation of all records, regardless of form or characteristics, mode of transmission, or state of completion. In accordance with 36 CFR 1222.32, all data created for Government use and delivered to, or falling under the legal control of, the Government are Federal records subject to the provisions of 44 U.S.C. chapters 21, 29, 31, and 33, the Freedom of Information Act (FOIA) (5 U.S.C. 552), as amended, and the Privacy Act of 1974 (5 U.S.C. 552a), as amended and must be managed and scheduled for disposition only as permitted by statute or regulation.

In accordance with 36 CFR 1222.32, Contractor shall maintain all records created for Government use or created in the course of performing the contract and/or delivered to, or under the legal control of the Government and must be managed in accordance with Federal law. Electronic records and associated metadata must be accompanied by sufficient technical documentation to permit understanding and use of the records and data.

LVAHCS and its contractors are responsible for preventing the alienation or unauthorized destruction of records, including all forms of mutilation. Records may not be removed from the legal custody of LVAHCS or destroyed except for in accordance with the provisions of the agency records schedules and with the written concurrence of the Head of the Contracting Activity. Willful and unlawful destruction, damage or alienation of Federal records is subject to the fines and penalties imposed by 18 U.S.C. 2701. In the event of any unlawful or accidental removal, defacing, alteration, or destruction of records, Contractor must report to LVAHCS. The agency must report promptly to NARA in accordance with 36 CFR 1230.

The Contractor shall immediately notify the appropriate Contracting Officer upon discovery of any inadvertent or unauthorized disclosures of information, data, documentary materials, records or equipment. Disclosure of non-public information is limited to authorized personnel with a need-to-know as described in the [contract vehicle]. The Contractor shall ensure that the appropriate personnel, administrative, technical, and physical safeguards are established to ensure the security and confidentiality of this information, data, documentary material, records and/or equipment is properly protected. The Contractor shall not remove material from Government facilities or systems, or facilities or systems operated or maintained on the Government's behalf, without the express written permission of the Head of the Contracting

Activity. When information, data, documentary material, records and/or equipment is no longer required, it shall be returned to LVAHCS control or the Contractor must hold it until otherwise directed. Items returned to the Government shall be hand carried, mailed, emailed, or securely electronically transmitted to the Contracting Officer or address prescribed in the [contract vehicle]. Destruction of records is EXPRESSLY PROHIBITED unless in accordance with Paragraph (4).

The Contractor is required to obtain the Contracting Officer's approval prior to engaging in any contractual relationship (sub-contractor) in support of this contract requiring the disclosure of information, documentary material and/or records generated under, or relating to, contracts. The Contractor (and any sub-contractor) is required to abide by Government and LVAHCS guidance for protecting sensitive, proprietary information, classified, and controlled unclassified information.

The Contractor shall only use Government IT equipment for purposes specifically tied to or authorized by the contract and in accordance with LVAHCS policy.

The Contractor shall not create or maintain any records containing any non-public LVAHCS information that are not specifically tied to or authorized by the contract.

The Contractor shall not retain, use, sell, or disseminate copies of any deliverable that contains information covered by the Privacy Act of 1974 or that which is generally protected from public disclosure by an exemption to the Freedom of Information Act.

LVAHCS owns the rights to all data and records produced as part of this contract. All deliverables under the contract are the property of the U.S. Government for which LVAHCS shall have unlimited rights to use, dispose of, or disclose such data contained therein as it determines to be in the public interest. Any Contractor rights in the data or deliverables must be identified as required by FAR 52.227-11 through FAR 52.227-20.

Training. All Contractor employees assigned to this contract who create, work with, or otherwise handle records are required to take LVAHCS-provided records management training. The Contractor is responsible for confirming training has been completed according to agency policies, including initial training and any annual or refresher training.

[Note: To the extent an agency requires contractors to complete records management training, the agency must provide the training to the contractor.]

D. Flow down of requirements to subcontractors

The Contractor shall incorporate the substance of this clause, its terms and requirements including this paragraph, in all subcontracts under this [contract vehicle], and require written subcontractor acknowledgment of same.

Violation by a subcontractor of any provision set forth in this clause will be attributed to the Contractor.

Q. INVOICES

The Contract Number and Purchase Order Number MUST be on the invoice.

The contractor must submit their invoices electronically via the use IPPS. A copy must be faxed (or emailed) to the COR, FAX 859-281-3867.

R. TRAINING REQUIREMENTS

All staff and administrators of Veterans' residential program will complete and document the following training, which is required prior to commencement of work and on an annual basis.

All Contractor employees and subcontractors under the contract are required to complete the NIH's On-line Security Awareness Training Course and the Privacy Awareness Training Course annually. Training can be accessed at <https://irtsectraining.nih.gov/publicUser.aspx>

Crisis Management – 4 hours a year, first training to be completed before commencement of services.

American Red Cross First Aid and CPR certification completed within 2 months of hire. Maintain certification during period of contract.

For new or existing contracts/business associates up for renewal, the contractor/business associate will provide proof of training to the contracting officer and the Contracting Officer's Representative (COR). Each year during the renewal process, the vendor of an existing contract will complete Privacy Training. Assistance will be sought from the contracting officer to ensure that the vendor is made aware of this requirement. A copy of the proof of training will also be maintained in the vendor's contract file.

Record Storage: Unless prohibited by federal statutes, at the conclusion of the first year of the contract and at least once per year thereafter through the completion of the contract, the contractor shall purge and/or destroy all VA Personally Identifiable Information (PII) from their systems by deleting the information from the hard drive and any other storage devices that are aged more than 12 months. At the conclusion of the contract, all remaining PII data shall be purged from the contractor's records. Information on tapes, paper documents or other media shall be erased and destroyed by burning or shredding. At each erasure during the contract cycle and contract determination, the Contractor shall certify in writing to the VA that the information has been destroyed.

Provisions that are incorporated by reference (by Citation Number, Title, and Date), have the same force and effect as if they were given in full text. Upon request, the Contracting Officer will make their full text available.

Points of Clarification from previous Solicitation in June 2025

1. Question # 1: The pre-solicitation suggests a capacity of 16-20 beds. Can you clarify if we can have two separate units: one with up to 16 beds for non-sex-offender veterans and another with 4 beds for veterans who are also sex offenders?

Answer # 1: Yes, you can have two separate units: one unit with up to 16 beds for non sex-offenders, and a separate unit with 4 beds for veterans who are on the sex offender registry. It is fine for these units to be at completely separate addresses. The units do not have to be near each other.

2. Question # 2: Are we allowed to reject any referrals?
Answer # 2: Yes, you are allowed to reject referrals, but the rejection cannot be based on a veteran's race, religion, or ethnicity.

3. Question # 3: Are we expected to have clinical, OT, or other professional services on site OR are we to collaborate/contract/refer to VA for medical, clinical, psychiatric, medication management services? The pre-solicitation mentions "therapeutic" services, however, defines this as psycho-educational/non-clinical further down in the document.
Answer # 3: You are not expected to have clinical, OT, or other professional services on site. All veterans housed in the transitional housing site will be VA healthcare eligible, so they will receive their healthcare through the VA. The therapeutic services provided on-site would be case management and/or psycho-educational in nature, helping veterans to develop life skills to work towards stability and complete tasks related to securing permanent housing.

4. Question # 4: Can you point us to the Social Determinants of Health Assessment?

Answer #4: The VA uses the ACORN screening tool, which can be found at https://www.va.gov/HEALTHYQUITY/docs/ACORN_Screening_Tool.pdf

5. Question # 5: Is the ISP (Individual Service Plan) a VA-created form or are we creating our own?

Answer # 5: The VA can provide a template of an Individual Service Plan, and you are allowed to customize the ISP for your site.

6. Question # 6: Certifications and Licenses – The pre-solicitation mentions meeting federal and state guidelines for residential programming. Can you point us to the specific regulations we should follow? Are we to refer to this document?

Answer # 6: In the Performance Work Statement, please refer to the following sections, which define what is required: Sections H. Conduct, K. Facility/Safety, L. Policy, M. Inspection and Acceptance, P. Quality Performance Standards, Q. Records, R. Invoices, S. Training Requirements. The performance work statement also designates that the facility must be ADA compliant for at least six residents.

7. Question # 7: Inspection Packet Requirements (Pg. 25–26): The Solicitation references the *Department of Veterans Affairs Health Care for Homeless Veterans CRS Programs Inspection Packet*. Can you clarify whether this packet must be completed and inspections conducted for all proposed sites **prior** to submission of the RFQ on June 11, 2025, at 2:00 PM CST?

Answer # 7: The inspection packet will be completed **AFTER** the RFQ is submitted on 6/11/25. VA staff members will come on site to complete the inspection once a contractor is selected.

8. Question # 8: Multiple Offers (Pg. 23, Item 8e): Are Contractors permitted to submit multiple RFQs in order to present pricing options for varying service capacities (e.g., minimum and maximum bed availability)?

Answer # 8: Yes, contractors are permitted to submit multiple RFQs in order to present pricing options for varying service capacities.

9. Question # 9: Level of Care and Client Needs: The Solicitation notes that the VA will assess clients prior to referral, but the required level of care for each individual remains unknown. Given that per diem pricing may vary depending on participants' physical, medical, emotional, verbal, and cognitive needs, can the VA provide guidance or a range of expected client needs to help inform pricing?

Answer # 9: In order for a veteran to reside at the transitional housing site, the veteran must be capable of completing all of their Activities of Daily Living independently. The contractor is not expected to provide OT, PT, speech or nursing care. The therapeutic services provided on-site would be case management and/or psycho-educational in nature, helping veterans to develop life skills to work towards stability and complete tasks related to securing permanent housing.

10. Question # 10: Supportive Recovery Model Requirements: Does the required Supportive Recovery Model need to be evidence-based or rooted in a promising practice, or can a provider submit a self-developed model that aligns with the stated objectives?

Answer # 10: The provider may submit a self-developed model that aligns with the stated objectives.

11. Question # 11: Training Content Requirements: For staff training topics such as crisis management, substance use, and mental health issues, can Contractors develop their own curricula based on current research and best practices, or must these trainings be created or approved by a specific agency?

Answer # 11: For staff training, contractors can develop their own curricula based on current research and best practices. The trainings do not have to be created or approved by a specific agency.

12. Question # 12: Gender Requirements for Living Quarters: Are Contractors expected to provide gender-neutral accommodations, or should living quarters be gender-specific?

Answer # 12: Men and women must have separate sleeping rooms. Ideally, it is preferable for men and women to be housed in separate buildings if possible.

13. Questions # 13: Social Determinants of Health Assessment Tool: Is the assessment tool for Social Determinants of Health provided by the VA or its partners, or is the Contractor expected to develop and implement a psychosocial assessment?

Answer # 13: The VA uses the ACORN screening tool, which can be found at https://www.va.gov/HEALTH/EQUITY/docs/ACORN_Screening_Tool.pdf. The contractor may use the ACORN screening tool if they wish.

14. Question # 14: Clinician Staffing Requirements: Is a licensed clinician required to be on-site or under contract? Alternatively, what level of support will the VA provide in coordinating clinical services such as mental health, physical healthcare, or housing navigation?

Answer # 14: A licensed clinician is not required to be on-site or under contract. However, the contractor is required to have competent staff who can provide therapeutic case management, helping veterans to develop life skills to work towards stability and complete tasks related to securing permanent housing. All veterans housed in the transitional housing site will be VA healthcare eligible, so they will receive their mental and physical healthcare through the VA. Staff members from the VA Homeless Team will meet with veterans to assist in housing navigation. The contractor's staff will be expected to also assist veterans with tasks that the veterans will need to complete in their housing process.

15. Question # 15: Drug Testing and Breathalyzer Costs: Are costs for breathalyzers and drug tests expected to be covered by the Contractor, or will these tests be administered and funded by the VA?

Answer # 15: The contractor will be responsible for paying the cost for breathalyzers and drug tests. The costs for these tests should be considered when the contractor determines the nightly rate they will charge.

16. Question # 16: Referral Denial Protocols: In situations where a Contractor deems a client inappropriate for the program, is the Contractor permitted to deny the referral?

Answer # 16: Yes, a contractor is allowed to deny referrals, but the denial cannot be based on a veteran's race, religion, or ethnicity.

17. Question # 17: Transportation Costs: Should the cost of bus passes or other forms of transportation be included in the Contractor's per diem rate?

Answer # 17: Yes, the cost of bus passes or other forms of transportation should be included in the Contractor's per diem rate.

18. Question # 18: Government IT Equipment (Pg. 16): The Solicitation references the use of Government IT equipment. Please clarify whether the VA will provide necessary hardware and software, or if procurement and maintenance are the responsibility of the Contractor.

Answer # 18: The contractor is responsible for the procurement and maintenance of all equipment, IT equipment, hardware and software. The contractor should consider those expenses when they calculate their nightly rate.

19. Question # 19: Employee Pre-Approval Requirements: Must all Contractor staff be approved by the VA **prior to RFQ submission**, or only prior to program implementation?

Answer #19: The contractor staff only needs to be approved prior to program implementation. The contractor staff does not need to be approved prior to RFQ submission.

20. Question # 20: Past Performance Submission (Pg. 23): Is there a specific evaluation rubric that the VA will use to assess Past Performance, or should Contractors submit a narrative that includes all required information as outlined on page 23 of the Solicitation?

Answer #20: There is not a specific evaluation rubric to assess past performance, as defined on page 23.

21. Question # 21: Pricing and Inflation (Pg. 29): Considering the pricing requirements outlined on page 29, is the Contractor permitted to include annual cost adjustments for inflation, or are prices expected to remain fixed for the duration of each contract year?

Answer # 21: The Contractor is permitted to include annual cost adjustments for inflation.